

Medications

metformin Oral
atorvastatin Oral

Medical History

Type 2 diabetes mellitus
Hyperlipidemia

Musculoskeletal History

None

Musculoskeletal Family History

None

Musculoskeletal Surgery

None

Surgical History

Appendectomy

Social History

Smoking status - Unspecified

Chief Complaints:

1. Left heel and arch pain, worse with first steps in the morning, 2 months.

HPI: This is a 69 year old male who:

Patient describes two months of left plantar heel pain that is most severe with the first steps in the morning and after periods of rest, improving somewhat with activity then worsening again by day's end. He works on his feet as a warehouse supervisor. No antecedent trauma. Has tried over-the-counter arch supports without relief.

Exam:

Comprehensive, Lower Extremity Neurovascular
Appearance: well nourished
Orientation: Alert and oriented to person, place, time.
Mood: mood and affect well-adjusted, pleasant and cooperative,
Right LE Pulses: normal dorsalis pedis pulse and good capillary refill
Left LE Pulses: normal dorsalis pedis pulse and good capillary refill
RLE Peripheral Sensation: intact to light touch throughout peripheral nerve distributions
LLE Peripheral Sensation: intact to light touch throughout peripheral nerve distributions

Ankle

Right Ankle Active ROM:

Dorsiflexion: 20 degrees.
Plantarflexion: 45 degrees.
Inversion: 35 degrees.
Eversion: 15 degrees.

Left Ankle Active ROM:

Dorsiflexion: 10 degrees.
Plantarflexion: 45 degrees.
Inversion: 35 degrees.
Eversion: 15 degrees.

Right Ankle Passive ROM:

Dorsiflexion: 20 degrees.
Plantarflexion: 45 degrees.
Inversion: 35 degrees.
Eversion: 15 degrees.

Left Ankle Passive ROM:

Dorsiflexion: 20 degrees.
Plantarflexion: 45 degrees.
Inversion: 35 degrees.
Eversion: 15 degrees.

Skin:

Right Ankle: skin intact, no rashes or lesions.

Skin:

Left Ankle: skin intact, no rashes or lesions.

Inspection:

Right Ankle: Normal alignment, no deformity, no tenderness, no warmth, no masses, no muscle atrophy, no crepitus

Inspection:

Left Ankle: tenderness at the plantar fascia insertion, no swelling

Strength:

Right Ankle Dorsiflexion: Strength: 5/5, normal muscle tone.
Right Ankle Plantarflexion: Strength: 5/5, normal muscle tone.

Strength:

Left Ankle Dorsiflexion: Strength: 5/5, normal muscle tone.
Left Ankle Plantarflexion: Strength: 5/5, normal muscle tone.

Stability:

Left Ankle: Stable

Special:

Left Ankle: Windlass test: positive

Tests

X-Ray Interpretation Foot

Diagnosis: Foot Pain, Left - M72.2

X-Ray Data:

Date: 06/03/2025

The following film(s) were done in our facility: left foot weight-bearing AP view and lateral view

Weight-bearing views of the left foot. Small plantar calcaneal spur. No stress fracture or acute osseous abnormality. Joint spaces preserved.

Impression: Plantar calcaneal spur consistent with chronic plantar fasciitis; no acute fracture.

Impression/Plan:

1. Foot Pain, Left

Plantar fasciitis, left foot (M72.2)

Associated diagnosis: Pain in left foot (M79.672)

Plan: Prescription.

diclofenac 75 mg tablet PO

Sig: Take 1 po bid x 3 weeks with food

Quantity: 42 Tablet Refills: 0

Follow up in 6 weeks

Note:

Clinical picture consistent with plantar fasciitis. Obtained weight-bearing foot films to evaluate for a heel spur and rule out stress fracture given his age and occupation. Recommended diclofenac, a night splint, calf and plantar fascia stretching, and supportive footwear.

Staff:

Marla Whitcomb, NP (Primary Provider) (Bill Under)

Electronically Signed By: Marla Whitcomb, NP, 06/03/2025 08:45 AM CDT